Name	: Mr. RADHESHYAM MITTAL		
Lab No.	: 485488209	Age	: 64 Years
Ref By	: Self	Gender	: Male
Collected	: 23/9/2025 11:32:00AM	Reported	: 24/9/2025 9:06:04AM
A/c Status	: P	Report Status	: Final
Collected at	: PAWAR DIAGNOSTIC Wadgaon Sheri pune,Punyanagari soc,Ayappa tample,Wadgaon Sheri PUNE 411014	Processed at	: LPL-PUNE LAB Ground floor, Anand Emerald, Sakore Nagar, Near Symbiosis University , Airport Road Pune-411014

MC-2762

Test Report

Test Name	Results	Units	Bio. Ref. Interval

3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1
4. Test conducted in Serum

LIPID PROFILE, SCREEN (CHO-POD)			
Cholesterol, Total	76.00	mg/dL	<200.00
Triglycerides	**183.00**	mg/dL	<150.00
HDL Cholesterol	**<10.00**	mg/dL	>40.00
Result Rechecked, Please Correlate Clinically.			
LDL Cholesterol, Calculated	30.40	mg/dL	<100.00
VLDL Cholesterol,Calculated	**36.60**	mg/dL	<30.00
Non-HDL Cholesterol	67	mg/dL	<130

Advice: LDL, Direct

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL& LDL Cholesterol.
2. Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
3. Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk.
4. Test conducted in Serum

Treatment Goals for Lipid lowering therapy (as per Lipid Association of India 2023)

ASCVD RISK CATEGORY	TREATMENT GOAL	
	LDL-C in mg/dL (Primary target)	NON HDL-C in mg/dL (Co-Primary target)

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.
Tel: 011-4988-5050, Fax: +91-11-2788-2134, E-mail: customer.care@lalpathlabs.com

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MC-2762

Test Report

Test Name	Results	Units	Bio. Ref. Interval
Low	<100	<130	
Moderate	<100	<130	
High	<70	<100	
Very High	<50	<80	
Extreme (A)	<50(<30 Optional)	<80(<60 optional)	
Extreme (B)	<30	<60	

ASCVD Risk Stratification & Treatment goals in Indian population

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

CSI Clinical Practice guidelines (2024) recommends in the absence of formal risk calculator for Indian population, only risk factors can be used for risk assessment. Standard Risk factors are:

- Smoking/tobacco use
- Hypertension
- Diabetes
- Family h/o Premature CAD (Men <55 years and women <60 years)

Risk Assessment*

Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
No standard risk factor	Presence of any one standard risk factor	• Presence of 2 or more standard risk factors with no manifest ASCVD • DM with 1 or more risk factor • Heterozygous Familial Hypercholesterolemia (HeFH) with no risk factor • Hypertension with one or more risk factor or with Target organ damage (TOD) • CKD- eGFR 30-59 ml/min	• ASCVD-CAD/PVD/CeVD • Imaging->50% lesion in any two major vessels • DM>20 years or multiple risk factors,TOD • HeFH -with ASCVD or RF • CKD- eGFR <30 ml/min	• ASCVD with recurrent vascular events. • ASCVD with HeFH & High Lp(a)

**** A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores***

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